Mock C&P Examination

Mental Disorders & Initial PTSD (MST) — PTSD · MDD · GAD · OCD · Substance Use Disorder secondary to PTSD

A full rehearsal of the VA Compensation & Pension mental-health examination, written around **your** claim file: the questions a VES examiner is likely to ask, in the order they usually come, what each question is really measuring, what your records already show the examiner, and how to answer it truthfully and completely.

Veteran	Marcus E. Klein · USAF SrA (E-4) · AFSC 2A333 F-15E Crew Chief · 4th Equipment Maintenance Squadron, Seymour Johnson AFB · 23 Nov 2009 – 22 Nov 2013 · Honorable
Exam	**Wednesday 21 October 2026 · VES (Maximus) · telehealth (video)** · spouse (Erica) attending
Claim posture	Supplemental Claim received 19 May 2026 (development letter 29 May 2026) · current combined rating **20%** (includes tinnitus 10%)
Contentions	PTSD due to in-service personal assault (MST, 2012) · MDD · GAD · OCD · SUD secondary to PTSD/MDD/GAD
Rating law in force	38 CFR 4.130 General Rating Formula (the 2022 proposed "5-domain" rewrite has **not** been finalized — your exam is under the current formula)
Prepared	18 October 2026 · from the full `va-disability/` corpus

THE RULE THAT GOVERNS EVERY PAGE

This document never tells you *what symptoms to have*. It tells you what the examiner is measuring and reminds you to give the **whole, accurate truth at its real depth** — worst days and typical days, with frequency, duration and consequences. Exaggeration is fraud (38 U.S.C. § 6103; 18 U.S.C. § 1001) and is also the fastest way to lose a true claim: validity testing and record comparison exist to catch it. Minimizing is the opposite failure, and it is the one most veterans actually commit.

Research and preparation assistance only — not legal or medical advice. Review with your VSO / VA-accredited representative. Contains personal health information; do not send to VA (internal prep material is never evidence).

TWO HEALTH AND SAFETY NOTES, BEFORE ANYTHING ABOUT THE CLAIM

You told us you are currently using opioids and looking for treatment. Combined with daily suicidal thoughts, that is a genuine overdose risk. Keep naloxone (Narcan — available without a prescription at pharmacies) where Erica can reach it. If thoughts turn active, call or text **988 and press 1** (Veterans Crisis Line), or text 838255. The VA can start medication treatment for opioid use disorder quickly — ask the Austin VA (or call 877-222-8387, the VA general health line) for same-day/urgent mental-health or SUD access. Getting into treatment does not hurt your claim; it helps you and it documents your condition.

Do not attend the exam impaired. An examiner who observes intoxication will document it, may stop the exam, and may attribute everything they see to the substance. Do not stop opioids abruptly on your own to "look better" either — unmanaged withdrawal is dangerous and will also distort the exam. If you are in withdrawal or have used recently, the honest move is to say so at the start. Talk to a prescriber about safe management before Wednesday if you can.

How to use this in the next three days

When	Do this	Pages
Sun 18 Oct	Read Part A (how the exam works) and the "Ten things that decide this exam" page. Fill in the **frequency worksheet** in the exam-day notes with your real numbers — with Erica, since she sees what you don't.	Part A, Ten Things, notes doc
Mon 19 Oct	Run Part B out loud with Erica as the examiner. Answer in your own words. Don't memorize — the goal is that nothing surprises you and nothing true gets left out. Mark any question where you went blank or minimized.	Part B
Tue 20 Oct	Read Part C (questionnaires) and Part D (the two mock reports). Re-run only the questions you marked. Do the technical check (camera, audio, private room, VES link). Re-read your own signed statements once so your account is consistent with what you filed.	Parts C–D, checklist (E)
Wed 21 Oct	Exam. Keep the one-page notes nearby for dates and numbers only. Afterward, write down what you were asked (Part E debrief).	Part E

Contents

1. **Part A — How this exam actually works** (who, what form, what the examiner decides, what the rater does with it, the 4.130 tiers mapped to your record, telehealth/VES specifics, the ten things that decide this exam)
2. **Part B — The mock examination** (≈150 questions and probes in real exam order, each with what's being assessed, what your file shows, and answer guidance)
3. **Part C — The questionnaires** (PCL-5, PHQ-9, GAD-7, suicide-risk screen, OCD screen, symptom-validity tests)
4. **Part D — Two mock examiner reports from the same facts** (a thin exam vs. a complete one — how answers become checkboxes)
5. **Part E — Exam-day checklist, debrief, scoring rubric and what to do after**

Part A — How this exam actually works

Knowing what the examiner is filling in is the single best antidote to a thin exam.

A1. Who you'll see and what they produce

- **Contract examiner, not your clinician.** VES (Veterans Evaluation Services, a Maximus company) is a VA contractor. Your examiner will be a licensed psychologist or psychiatrist — sometimes a psychiatric nurse practitioner or LCSW for some non-initial exams, but an *initial* PTSD exam requires a psychologist or psychiatrist. They have no treatment relationship with you, and VES's own FAQ describes the exam as the same exam a VA facility provider performs.
- **The form.** Because PTSD is not yet service-connected, expect the **Initial PTSD Disability Benefits Questionnaire** (VA Form 21-0960P-4 structure), which also captures your other mental diagnoses. Its companion, the Mental Disorders DBQ (21-0960P-2), has the same core items. Either way the examiner fills the same skeleton: diagnoses → differentiation between diagnoses → **one** overall impairment checkbox → history (pre-, during and post-military; social, occupational, mental-health, legal, substance) → stressor(s) and Criterion A → PTSD criteria B–H → symptom checklist → behavioral observations → other symptoms → competency to manage funds → remarks/testing → medical opinion(s).
- **Time.** Plan for roughly 60–90 minutes; initial PTSD exams with records review and testing can run longer, and some examiners split testing into a separate session. A 20–30 minute exam on a claim this complex is a red flag (see Part E).
- **Records.** The examiner usually reviews your claims file (VBMS) beforehand: your 21-0781 and personal statement, the Reyes and Poquette buddy statements, Erica's statement, Cherie Mills's letters, your VA records (including the 2015 Jennings CBOC visit), and whatever private treatment records were submitted. **Assume they have read the worst page in your file.**

A2. The questions the examiner must answer for VA

Question on the DBQ / opinion request	Why it matters for you
1. Does the veteran have PTSD (DSM-5) — and which other mental disorders?	Cherie's letters are the main clinical PTSD diagnosis. The 5/15/2026 Recover eval also mentions PTSD but attributes it to "childhood trauma and military experiences" — a competing-cause phrase the examiner may raise. This exam is the first VA-side diagnostic look. A full, consistent account of criteria B–E across the past month (and since 2012) is what gets the diagnosis confirmed.
2. Does the stressor meet Criterion A, and is it an in-service personal assault (MST)?	An incapacitated sexual assault meets Criterion A. The DBQ then asks the examiner to describe "markers" that may substantiate it (38 CFR 3.304(f)(5)).
3. **Marker opinion:** does the evidence indicate the personal assault occurred?	VA may ask the examiner whether the record's behavior changes indicate an assault occurred. Your markers (A5) are documented by three lay sources. The examiner may not treat the lack of an in-service report as proof it didn't happen (*AZ v. Shinseki*).

4. Nexus: is each diagnosis at least as likely as not caused or aggravated by the in-service stressor?	This is where the pre-service history (age-18 self-reports), childhood factors and autism can be raised as competing causes.
5. Is SUD secondary to PTSD/MDD/GAD — and can SUD symptoms be separated from psychiatric symptoms?	Primary drug abuse isn't compensable; drug abuse *secondary to* a service-connected condition is (*Allen v. Principi*). If the examiner can't separate the effects, reasonable doubt attributes them to the service-connected condition (*Mittleider*). Active use makes this the most contested item on Wednesday.
6. **One checkbox**: the overall level of occupational and social impairment from all mental diagnoses.	This is the item the rating most often follows. Everything else in the exam feeds it.
7. Is the veteran capable of managing his own financial affairs?	An honest "no" can lead to a VA incompetency proposal and a fiduciary (38 CFR 3.353). See A8.

A3. What the rater does with the exam

The examiner does not assign the percentage — a rater (RVSR) does. The rater must evaluate "all the evidence of record that bears on occupational and social impairment rather than solely on the examiner's assessment … at the moment of the examination" (38 CFR 4.126(a)). Cherie's signed letters, Erica's statement, the DASS-21/WHODAS, and the treatment records are all weighed. In practice, though, the examiner's single impairment checkbox carries the most weight. A thin exam is fixable afterward, but it costs months. The goal Wednesday is an exam that *captures* what is already in your file and what is true today.

Two rules shape the 70-versus-100 decision:

- **The symptom lists are examples, not checklists** (*Mauerhan v. Principi*). What counts is symptoms of similar severity, frequency and duration that actually *cause* the level of impairment (*Vazquez-Claudio v. Shinseki*). Frequency, duration and consequence are what the examiner needs from you, not labels.
- **100% requires total occupational AND total social impairment**, and VA may not rate solely on social impairment (4.126(b)). Since you are unemployed, the occupational side is the stronger prong. The **social prong — with a marriage, AA, and a weekly outing on the record — is where 100% is won or lost.**

A4. The 4.130 tiers, mapped to what is already in your file

Tier	Regulatory language (abbreviated)	Where your record already speaks to it
100% Total occupational and social impairment	Gross impairment in thought/communication; **persistent delusions** or hallucinations; grossly inappropriate behavior; persistent danger of hurting self/others; **intermittent inability to perform ADLs (incl. minimal personal hygiene)**; disorientation; memory loss for names of close relatives/own occupation/own name.	**ADLs/hygiene:** Erica washes and picks your clothes, reminds you to brush your teeth, and manages appointments (Cherie's letters); her 7/12/2026 statement documents ADL caretaking; you have also reported she feeds you and cuts your hair; WHODAS self-care 62.5%. **Belief content:** Cherie's letter describes evidence-resistant fear of being fired/failing as "paranoid delusions" — the examiner will probe this. **Not claimed and not your experience:** disorientation, hallucinations, memory loss for own name/relatives — don't drift toward them.
70% Deficiencies in most areas (work, school, family, judgment, thinking, mood)	**Suicidal ideation**; **obsessional rituals** interfering with routine; **near-continuous panic or depression** affecting independent function; impaired impulse control; neglect of hygiene; difficulty adapting to stress; **inability to establish and maintain effective relationships**.	Daily passive SI (personal statement; Cherie; Erica); OCD (F42.2 coded Starlite 2023–24); panic "more mornings than not" (Cherie); DASS-21 all three scales "Extremely severe" (6/9/2026); repeated job losses; isolation outside the marriage.
50%	Reduced reliability and productivity: panic attacks more than weekly, memory problems, impaired judgment, disturbed motivation/mood, difficulty with work/social relationships.	The floor a thin exam lands on if frequency/consequence detail is missing.
30% / 10% / 0%	Generally functioning satisfactorily…	Not consistent with the record.

Combined-ratings math with your current 20% (38 CFR 4.25; same result whether the 20% is one 20 or two 10s): mental health 50 → 60% combined · mental health 70 → 80% combined · mental health 100 → 100%. There is no 90% mental-health tier. A later 50% sleep-apnea rating on top of MH 70 + your 20% ≈ 88 → 90%, not 100%.

A5. Your MST "markers" — what the examiner can point to

Marker category (3.304(f)(5))	Evidence in your file
Behavior change after the assault	Reyes: "noticeable change" in summer 2012. Poquette: after ~8–9 months of daily contact (met 2011) "Marcus started to become distant"; he confronted you and you disclosed.
Substance abuse	Escalating alcohol 2012–13; late-2012 alcohol-related disciplinary action; post-service escalation to opioids/benzodiazepines; 9+ treatment episodes 2016–2026.
Depression / panic / anxiety without an identifiable cause	Insomnia treated in service (Ambien/Restoril — not yet corroborated by STRs); 2013–15 deterioration (depression, panic, hypervigilance, isolation).

Unexplained economic or social change	Poquette: separated with "a good amount of money saved," then about a year later asked him for money; dropped contact with the friend group.
In-service disclosure to peers	Poquette (in-service by narrative context); Reyes ("late 2013, while we were both still serving").
What you did **not** claim (consistent, credible)	No transfer request; duty performance held up (21-0781 "changes in performance" left unchecked); BMT Honor Graduate, Good Conduct Medal — a clear pre-assault baseline.

A6. Telehealth with VES — practical rules

- **Room:** private, quiet, door closed, good light on your face, phone/laptop on a stable surface at eye level, charger plugged in. Have the VES phone number (877-637-8387) and your appointment confirmation handy in case the video drops. Log in 10–15 minutes early — VA warns late arrival can cancel an exam.
- **ID:** expect to show photo ID on camera and confirm your identity and location (they need to know where you are for emergencies).
- **Erica:** family members may generally sit in only with the examiner's approval and may not take part unless invited. At the start, say: "My wife is here; she helps me with daily things and with remembering. Is it okay if she stays, and could she add anything at the end if you want collateral information?" If the examiner asks her to step out, especially for the assault questions, **agree without argument** — refusing can end the exam. Ask whether she can speak with them for a few minutes afterward.
- **Recording:** do not record. Contractor telehealth exams are generally not recorded, and recording without permission can end the exam. Erica can write notes *after* the call.
- **Notes:** VA says you don't need to bring anything. A short sheet of **dates, medication names and your frequency numbers** is reasonable — glance at it, don't read answers from it. If asked, say plainly: "I wrote down dates so I'd get them right." Scripted-sounding answers hurt credibility.
- **What the camera sees is the mental-status exam.** The examiner will record appearance, grooming, eye contact, speech, affect and behavior. If Erica helped you shower, shave or dress for the call, **say so** — otherwise "well-groomed" goes on the DBQ and quietly undercuts your strongest 100%-tier fact.

A7. The examiner's mindset — and the traps that come with it

- **They test consistency, not just severity.** Every answer is compared against your filed statements and records. The biggest risk is not a bad symptom; it's an answer that contradicts something you signed without an explanation.
- **Open questions, then pointed ones.** "Tell me about your mood" comes first, then "How many days in the last two weeks…". Give the concrete number when you get the pointed version.
- **"How are you doing?" is not small talk.** "Fine" or "good" can be charted as self-reported mood. Answer it truthfully.
- **Telescoping.** Examiners write in present tense ("He reports panic attacks a few times a month") from whatever you said last. If something has changed, say when.

- **Best-day bias.** The exam is a planned appointment, often on a better day, with Erica's help. Describe the typical week and the worst stretches as well as today.
- **Validity testing** (Part C) catches both over-endorsement and inconsistent responding. Read every item; answer each honestly.

A8. Competency to manage VA funds — know the consequence before you answer

The DBQ asks whether you can manage your own financial affairs. If the honest answer is that Erica manages money because of your substance use (her statement references roughly $30,000 spent on drugs), say so. That is real evidence of severity. Know what it can lead to: VA may **propose** an incompetency rating (38 CFR 3.353) — it needs a definite medical statement, gives you notice and a chance to respond, and resolves reasonable doubt *in favor of* competency — and, if finalized, VA's fiduciary program (38 CFR Part 13) appoints someone (often a spouse) to receive payments. That is not a punishment and it does not reduce the rating. But it should not be a surprise, and you should never shade the truth either way to avoid it.

The Ten Things That Decide This Exam

Ranked by how much each one moves the result. Every item maps to questions in Part B.

#	Issue	The risk on Wednesday	The truthful counterweight
1	**Total *social* impairment vs. the marriage**	"Married, attends AA, weekly social outing" → examiner checks 70%, not 100%.	Describe exactly what the marriage is now (caregiving, near-divorce, couples counseling) and exactly what exists outside it: who you talk to, how often, whether any relationship is reciprocal, and what the AA attendance and the weekly outing actually consist of (who decided, how you function there, what it costs you). Don't inflate; don't omit.
2	**ADL / hygiene dependence**	Not asked, or "well-groomed" charted from the video.	Volunteer it if not asked: who does what, how often, what happens without Erica. Say whether she helped you get ready for this call.
3	**The fear of being fired / failing**	Examiner tests whether it's a fixed false belief or anxiety; a vague answer reads as anxiety, and a labeled answer ("I have delusions") reads as coached.	Describe content, how certain you are, what happens when someone gives you evidence otherwise, how long it lasts, and what you do because of it (the overwork → collapse cycle). Let the examiner choose the clinical word.

4	**Suicidal ideation — the "denies SI" entries**	Three different SI statements in May–June 2026, plus a Starlite ASAM self-harm checkbox.	Give your real current frequency and nature, and explain plainly that you denied SI at intakes to avoid an involuntary hold (Cherie documents this).
5	**Ten years of denying trauma in records**	2015 VA: "denies MST," PC-PTSD 0/4; 2018 Victory ×2; Starlite "no military trauma."	Your own explanation: denial and avoidance, shame, not seeing it as "assault," distrust of intake staff, fear of holds. Stay consistent with your signed statement that you told a Starlite therapist in 2022. Delayed disclosure is common in male MST, and a missing report isn't negative evidence.
6	**Active opioid use**	"Symptoms are substance-induced"; SUD seen as primary or willful.	Be honest about current use. Describe which symptoms persist when you are abstinent or in residential treatment (if true), the order things happened (assault → symptoms → self-medication), and that you're seeking treatment.
7	**Pre-service history and other causes**	Age-18 anxiety/depression and kratom self-reports; DUI at 18, paraphernalia arrest at 20; childhood neglect; autism; Recover eval "childhood trauma and military experiences."	Answer honestly and without defensiveness. Then contrast the baseline (enlisted, BMT Honor Graduate, Good Conduct Medal, functioning, friend group) with the post-2012 change. Caused-vs-aggravated is the examiner's call.
8	**Perpetrator status error and the missing Goff statement**	Your 21-0781 says "female service member"; Reyes ("wife of a coworker"), Poquette ("one of our friend's wife"), your personal statement and the CPT worksheet say otherwise. Goff was promised and never submitted.	Correct it plainly: she was a coworker's wife (a civilian as far as you know), and the form entry was an error. Goff agreed but never sent it.
9	**Work history read as capacity**	"Obsessive overworking," "valued employee," 2024 "gainfully employed," Poquette's "he has improved."	Sequence: worked through effort, then burnout and relapse, then job loss (2018 near-loss, 2023, Dec 2024, June 2026 PIP/severance), then you left the field. The overwork was driven by fear and ended in collapse.
10	**Thin exam / best-day presentation**	A short exam that never gets frequencies, the worst stretches, or the social prong.	Use the closing question to add anything missed (Part B §B13). Afterward, request the DBQ and challenge an inadequate exam.

Part B — The Mock Examination

Real exam order. Each block: the examiner's question and likely follow-ups → what is being assessed → what your file already shows → how to answer truthfully and completely.

HOW TO REHEARSE WITH THIS

Have Erica read the **bold question** and the italic follow-ups. Answer out loud, in your own words, from memory. Then read the guidance together and ask: *Did I give a number? A duration? A consequence? A real example from the last month? Did I leave anything true out because it was embarrassing?* Do not memorize phrasing — examiners recognize rehearsed language, and it costs credibility.

Key: Assessing = what the examiner is measuring · Your file = what they may already have read · Guidance = how to answer well · Trap = the specific way this question goes wrong. HIGH = among the Ten Things.

B0 · Opening (≈5 minutes)

B0.1 "Can you confirm your name, date of birth, and where you are right now? Is anyone with you?"

ASSESSING Identity, location (for emergencies), privacy, and orientation (the first item of the mental-status exam).

GUIDANCE DOB 05/30/1989. Give your Austin address. Introduce Erica and ask whether she may stay (A6). If any document the examiner reads shows "30 May 1998" or "Kline," that's a clerical error in Cherie's first letter — just correct it.

B0.2 "How are you doing today?" HIGH

"How did you sleep last night? Did you have any trouble getting here / setting up?"

ASSESSING Self-reported mood, which is often charted verbatim ("Veteran reports mood is 'okay'").

GUIDANCE Answer the real question. If you're anxious, exhausted or barely slept, say so. If Erica had to get you up, showered or set up for the call, say that now — it heads off a "well-groomed, cooperative, punctual" impression.

TRAP Reflexive "good, thanks." That word ends up in the report.

B0.3 **"Have you used any alcohol, drugs, or medications today that could affect this interview? When did you last use opioids?"** HIGH

ASSESSING Validity of the exam itself, and whether what they observe is substance-induced (intoxication or withdrawal).

YOUR FILE Relapse documented June 2026 (opioid pain pills and kratom, Suboxone discontinued); Harmony Junction April–May 2026. You report current opioid use.

GUIDANCE Tell the truth, including the last time you used and whether you are in withdrawal. Add, briefly and truthfully, that you are trying to get back into treatment and what steps you've taken (calls, referrals, waitlists).

TRAP Denying use that the examiner then sees on camera, or that conflicts with records — one false answer can discredit every true one.

B0.4 **"Do you understand the purpose of this exam? I'm not your treating provider, and what you tell me goes into a report for VA."**

GUIDANCE Yes. You can also say if you'd prefer not to go into detail about the assault itself. The C&P Clinicians' Guide discourages examiners from pressing for a detailed account of MST, but they must still establish what happened at a basic level (B4).

B1 · Records review, diagnoses, current treatment

B1.1 **"What mental-health diagnoses have you been given, and by whom?"**

YOUR FILE PTSD, MDD, GAD, OCD, SUD — Cherie Mills, LCSW (treating since 6 Feb 2023; two signed letters). MDD (F33.1) and GAD (F41.1) coded across Starlite 2023–24, Harmony Junction 2026, and the FMLA "Recover" evals (May 2026). OCD (F42.2) coded at Starlite Jul–Aug 2023 and Jul–Aug 2024. Opioid use disorder (severe), sedative/hypnotic use disorder. One unexplained F15.20 "amphetamine-type SUD, severe" at Harmony Junction. Self-reported autism spectrum. The 5/15/2026 Recover eval references PTSD ("childhood trauma and military experiences"); the 5/14 version codes only GAD/MDD.

GUIDANCE List what you've actually been told, by whom, and roughly when. If F15.20 comes up and you don't know where it came from, say so and that you'd have to ask the facility. Don't invent an explanation. (Harmony's own summary mentions "stimulants" once in passing — if you did use stimulants, say so honestly.)

B1.2 **"Are you in mental-health treatment right now? Who, how often, since when? Any medications?"**

"When did you last see Ms. Mills? What do you work on in therapy? Do you take everything as prescribed?"

ASSESSING Current treatment intensity (a severity indicator), medication adherence, and whether the psychiatric picture has been treated.

YOUR FILE CPT with Cherie Mills since Feb 2023; VA MH consult 6/17/26; Community Care referral to WellMind PLLC 6/21/26; Suboxone stopped June 2026; long-term Eliquis after the 2022 DVT/PE (verify current).

GUIDANCE Be precise about what is current today versus what was planned. If the VA/WellMind referral didn't result in visits, say so and why. Have your medication list on your notes page.

B1.3 **"Walk me through your hospitalizations and residential treatment."**

YOUR FILE Acadiana (Sunset, LA) 2016 and May 2017 · Victory Addiction Recovery Center 2018 (detox 8/25–9/1; residential 9/4–10/4; IOP to 11/20) · Starlite Jan–Feb 2022, Jan 2023, Jul–Aug 2023, Jul 30–Aug 13 2024, Dec 26 2024–Jan 3 2025 (left AMA) · Ocean's (San Clemente) ~3 weeks April 2025 · Harmony Junction Apr 17–May 11 2026, then Zoe PHP (left early).

GUIDANCE You don't have to recite every date — say "at least nine treatment episodes since 2016" and give the most recent ones accurately. Use your notes for dates. If you left AMA, say why truthfully (Jan 2025: the remote job offer; Zoe PHP: you couldn't handle the coed housing). **Harmony Junction was 24 days** by the discharge dates (your statement said 25 — trivial; don't argue it).

B2 · Pre-military history

> **Why this section matters more than it looks:** the examiner is establishing your **baseline** and looking for **alternative causes**. Your records contain an age-18 self-report of anxiety/depression treatment, kratom use from 18, a DUI at 18 (later expunged), a paraphernalia arrest at 20, both "pretty good childhood" and "neglected most of childhood," and self-reported autism. Answer every one honestly. Hiding a pre-service fact that is sitting in the Starlite record is far more damaging than the fact itself. Being sound at entry is presumed unless something was noted on your entrance exam (38 U.S.C. § 1111), and aggravation is a separate route — those are legal questions for the rater, not things you need to argue.

B2.1 "Tell me about growing up — family, home life, school."

"Any abuse or neglect? How did you get along with your parents? Any learning problems, special education, fights, suspensions? Friends growing up?"

ASSESSING Developmental baseline, childhood trauma (a competing cause), and social functioning before service.

YOUR FILE Starlite has both "'pretty good' childhood" (2022) and "neglected most of childhood" (2023), plus "on the spectrum" since childhood. The 2023 Starlite biopsychosocial attributes distress to childhood neglect, autism and work stress, with no MST mention. Paternal criticism appears once as a relapse trigger.

GUIDANCE Give your honest, considered account. If both descriptions feel partly true (a stable surface, emotional neglect underneath), say exactly that. Then describe how you actually *functioned* before service: friends, school, work, sports. The examiner needs a baseline to measure the 2012 change against.

TRAP Defensiveness ("my childhood was perfect"). Records already say otherwise, and an examiner can read that as unreliability.

B2.2 "Did you have any mental-health problems or treatment before you joined?" HIGH

"Your records say you were diagnosed with anxiety and depression around 18 — tell me about that. Any medications? Counseling? How long?"

YOUR FILE 2018 Victory (age-19 self-report) and 2022/Jan 2023 Starlite (age-18 self-report) of anxiety/depression treatment before enlisting. Your entrance exam (in the STRs) is the controlling record — check whether it noted anything if you have a copy.

GUIDANCE Say exactly what happened — who, what, how long, how severe — and how you functioned then (graduated, worked, passed MEPS, enlisted, BMT Honor Graduate). If it was brief or mild, describe it accurately. Then, if true, describe what changed after 2012 in kind and severity: nightmares, hypervigilance, shame, panic, SI, substance escalation. That contrast is the core of an aggravation-or-new-onset finding. The examiner decides which it is; your job is accurate facts.

TRAP Contradicting the record ("I never had any problems before"). The Starlite entry is in your file.

B2.3 "Any alcohol or drug use before the military? Any legal problems?" HIGH

"Your records mention kratom at 18 and a DUI. Any other arrests?"

YOUR FILE Starlite: substance use "began at the age of 18 with Kratom"; arrested at 18 (DUI, later expunged) and 20 (paraphernalia). Victory 2018 lists past charges. Your signed personal statement said you didn't recall opioids before 2011. The corpus records that you separately took a prescribed medication before service that you didn't know was an opioid, without misuse — and that this was addressed with your VSO/attorney.

GUIDANCE Answer completely and plainly: what you used, how often, whether it caused problems then, and the legal history. If you distinguish experimental or prescribed use before service from the disorder that developed after 2012, say so in your own words — but only as far as it's genuinely true. If the examiner raises the personal-statement wording, give the honest explanation (you didn't think of that use as "opioid use," or you misremembered — whichever is true) and don't argue.

TRAP Minimizing a documented arrest or the kratom history. That is the single easiest credibility hit in your file.

B3 · Military history

B3.1 **"Why did you join? How was basic training, tech school, your first duty station?"**

YOUR FILE Lackland BMT (Honor Graduate) → Sheppard tech school → Seymour Johnson AFB, 4th EMS (DD-214; Reyes's form says "4th Maintenance Squadron," which is a minor error on his part), F-15E crew chief. Poquette: you trained him on F-15s, let him stay at your apartment, "always there to help."

GUIDANCE Describe the pre-2012 you concretely: performance, friendships, the weekend friend group, helping others. This is the baseline that makes the change visible.

B3.2 **"Any disciplinary actions, Article 15s, LORs, performance problems? Any injuries or medical treatment in service?"**

YOUR FILE Late-2012 alcohol-related disciplinary action (connected to an airman's DUI arrest). Ankle fracture — "May 2011" in your statement but "2012" in the 2015 VA intake (unresolved) — at Wayne Memorial, with morphine and oxycodone. Insomnia treated with Ambien/Restoril (not yet confirmed by STRs). Duty performance held up.

GUIDANCE Answer accurately and give dates only as precisely as you actually know them ("2011, I believe — my records conflict"). The disciplinary action is a *marker* (escalating drinking after the assault), so describe it factually.

B3.3 **"Did anything change for you during your service — how you felt, slept, drank, got along with people?"**

ASSESSING Behavioral markers for the personal-assault stressor (3.304(f)(5)) — the examiner may be asked for an opinion on them.

GUIDANCE Describe the before/after concretely: when drinking increased, sleep problems and the sleeping pills, pulling away from the friend group, panic, anger, eating or exercise changes. These are what Reyes and Poquette observed. Mention that your duty performance held up if that's true — consistency is credibility.

B4 · The stressor (MST)

> For Criterion A the examiner needs to know **that** a qualifying event occurred, roughly **when**, and **how you responded** — not a graphic account. You can say "I'd rather not go into the details, but I can tell you what happened in general." If you choose to say more, everything must match what you've filed: **summer / mid-to-late 2012**, **a coworker's wife**, at a party, while you were **incapacitated**; preceded by harassment from early 2012.

B4.1 "What is the event or events you believe caused your symptoms?" HIGH

"Approximately when? Where? Who was involved — another service member? Were you drinking? Did you feel your life or safety was threatened? What did you feel right after?"

YOUR FILE 21-0781 Item 9 says "same female **service member**." Reyes ("wife of a coworker"), Poquette ("one of our friend's wife"), your personal statement and the CPT worksheet all describe a coworker's wife. Cherie: "an incapacitated assault … male survivors of female-perpetrator assault."

GUIDANCE State it in your own words at the level of detail you can tolerate. If asked whether she was a service member, correct the form: she was a coworker's wife, a civilian as far as you know, and the "service member" entry on the 21-0781 was a mistake. Describe your immediate reaction honestly (confusion, shame, numbness — whatever is true).

TRAP Adding new details on the spot that appear nowhere in your filings. Any new detail invites "why is this the first time we're hearing this?"

B4.2 "Did you report it to anyone at the time — command, SARC, medical, chaplain, police?"

GUIDANCE No official report, and say why truthfully. Your personal statement says you'd never heard of a male airman reporting a female perpetrator and believed "real men" couldn't be assaulted by women. Also mention the fear, the shame, and that she was a friend's wife in your social circle — whatever is true.

B4.3 "Who did you tell, and when?"

YOUR FILE Poquette: he noticed you becoming distant after ~8–9 months of daily contact from 2011, confronted you, and you told him. His signed statement gives no date. Reyes: "late 2013, while we were both still serving" (you separated 22 Nov 2013). The 21-0781 promised a statement from Liam Goff, which was never submitted.

GUIDANCE Name who you told and roughly when, only as precisely as you remember. If Goff comes up: he was going to write one but never sent it.

B4.4 "Your 2015 VA visit says 'Patient denies experiencing military sexual trauma,' and your PTSD screen was negative. Your 2018 intakes say you denied any trauma. Help me understand that." HIGH

"And at Starlite you checked 'no military trauma' — but in January 2022 you endorsed an unwanted sexual experience. Why the difference?"

YOUR FILE 2015-09 Jennings CBOC: explicit denial, PC-PTSD 0/4, PHQ-2 0. Victory 8/25/18 and 10/1/18: denied abuse/trauma. Starlite Jan 2022: LEC-5 endorsed "other unwanted or uncomfortable sexual experience" (and some other items), nursing note "sexual abuse: yes," narrative "denies trauma," EMDR during that stay. Later Starlite admissions: "none reported." **Your signed personal statement says you disclosed the MST to a Starlite therapist in 2022** (it names "Melissa Robbins"; the Starlite records name "Robbin Lynch, MS, LCDC"). Cherie's letter explains that male survivors of female-perpetrated assault often don't label it "assault" and minimize it, and that you minimized at intakes to avoid holds.

GUIDANCE This is the question to have thought through, not scripted. Give your real reasons in your own words — for example: you didn't think of it as assault, shame, it was a 5-minute intake with a stranger, you were there to detox, saying "trauma" or "SI" risked a hold, you weren't ready until you trusted Cherie. **Stay consistent with your signed statement:** if you did tell a Starlite therapist in 2022, say so, with the name as best you remember it, and explain that the intake screens still said no. If the 2022 LEC-5 comes up, explain what was true then (you checked the closest box without wanting to talk about it, or whatever actually happened). Calm and matter-of-fact beats defensive.

TRAP Claiming you disclosed when the record shows you didn't, or getting angry. Silence in records is not evidence it didn't happen — but a contradiction you can't explain is a credibility problem.

B4.5 "Any other traumatic events — before, during, or after service?"

YOUR FILE The Jan 2022 LEC-5 also endorsed items 1, 3, 6 (physical assault), 12 and 13 as "happened to me." None of these is narrated anywhere in the claim.

GUIDANCE Answer honestly. If there are other events, name them briefly, with dates and whether they bother you now. The examiner has to consider them, and concealing them is worse than disclosing them. If you don't remember what those 2022 checkboxes referred to, say that.

B4.6 "Which event bothers you most now? When you have nightmares or intrusive memories, what are they about?"

ASSESSING Whether symptoms are linked to the claimed stressor (the DBQ asks this per stressor).

GUIDANCE Be specific and truthful about content. If intrusive content is about the assault, its aftermath, or the shame and self-blame, say so. If some nightmares are about other things, say that too.

B5 · Post-military occupational history

Order of events (verify against your own records before Wednesday): separated 11/2013 → 2013–15 deterioration, money gone in about a year → B.S. Computer Science, McNeese, Dec 2016 → tech career → 2018 near job loss, saved by treatment → 2023 job loss, ~6 months unemployed, nearly lost the home → Jul–Aug 2024 Starlite discharge lists "gainfully employed" → **Dec 2024 laid off** ("recently laid off" at Starlite readmission) → remote CTO offer, left Starlite AMA Jan 2025 → spring 2026 residential, FMLA → **June 2026 PIP / performance loss, negotiated severance** → left the software career.

B5.1 "Are you working now? What was your last job, and why did it end?" HIGH

"Were you fired, laid off, or did you quit? Any PIPs, warnings, attendance problems? Did you take FMLA or disability?"

ASSESSING The occupational prong, and specifically whether job losses were *caused by* symptoms.

GUIDANCE Give the sequence with the *mechanism*: what symptoms did at work (missed days, panic, couldn't concentrate, the fear of being fired driving 18-hour days, collapse, relapse, residential, FMLA, PIP). **If you are doing any work at all now** — even informal or for family — disclose it, with hours, pay and accommodations (work in a protected setting such as a family business *can* be found "marginal" rather than substantially gainful under 38 CFR 4.16(a), on the facts — but only if it's described accurately).

TRAP "I decided to leave tech" with no context reads as a voluntary career change. If symptoms drove the decision, say how.

B5.2 "You held senior tech jobs — your records say 'gainfully employed' in 2024 and a CTO offer in 2025. How does that fit with being unable to work?" HIGH

YOUR FILE Cherie's letter: you "fluctuate between obsessive overworking and fully shutting down," and employers showed you "evidence regarding [your] value in the role." Poquette: "in the past couple of years he has done a lot to better his life."

GUIDANCE Describe the cycle as it actually happened: what the overworking looked like (hours, the fear behind it), what the crash looked like (days unable to function, relapse, treatment), how many times it repeated, and how long each job lasted. Explain the timing — you were employed in mid-2024, and the losses came Dec 2024 and June 2026. Past capacity doesn't contradict a current inability; the dates show the decline.

B5.3 "If someone offered you a simple, low-stress job tomorrow, could you do it? What would stop you?"

ASSESSING Residual capacity — the question examiners use to separate "can't do my old job" from "can't sustain any work."

GUIDANCE Don't answer with a reflexive "no." Walk through a real workday honestly: getting up, hygiene without Erica, getting there, being around people, concentrating, panic, the fear of failing, reliability over weeks. Be accurate. If you could manage some things, say which, and what would break down and how fast.

B6 · Social and marital functioning (decides 100%)

The examiner will form the social-impairment judgment here. On the record are your marriage (since Nov 2022), AA attendance, a commitment to "one social outing a week," occasional contact with old service friends, and couples counseling since 2020. The 100% tier needs **total** social impairment; 70% is "inability to establish and maintain effective relationships." Neither inflating ("I have no one") nor reflexively normalizing ("yeah, I go to meetings, I'm married") is accurate. Give exact facts: who, how often, what kind of contact, whether it's two-way, who initiates, and what it costs you.

B6.1 "Tell me about your marriage." HIGH

"How do you two get along? Do you do things together for fun? Any separations, talk of divorce, counseling? What does she do for you day to day? What do you do for her?"

YOUR FILE Met 2018 in 12-step recovery; married Nov 2022. Cherie: strain "to the point of near-divorce"; wife manages hygiene, clothing and appointments. Erica (7/12/2026): caretaker role, isolation, intimacy impairment, SI. Couples counseling on and off since 2020.

GUIDANCE Describe the relationship as it actually functions now, including the last question — what you are able to give back. If Erica functions mostly as your caregiver (hygiene, meals, reminders, appointments, money), say what that looks like on a normal day and on a bad day. Include the near-divorce periods and the intimacy problems if true. Don't disparage her, and don't describe a warmth or reciprocity that isn't there.

TRAP "She's my rock, we're great" — said out of loyalty, it is charted as an effective relationship.

B6.2 "Besides your wife, who do you talk to? Friends? Family? When did you last see a friend?" HIGH

"Do you have a best friend? Do you keep in touch with anyone from the Air Force? Your brother? Your parents? Neighbors?"

YOUR FILE Cherie (v2 letter, as evaluated): isolation for "6 to 7 years." Poquette: "stayed in touch here and there." WHODAS: "making new friends" = cannot do; getting-along 70%; participation 75%.

GUIDANCE Go person by person: name the relationship, when you last had contact, who initiated it, and what it consisted of (a text, a call about the claim, a holiday). If you reached out to Reyes and Poquette only to ask for statements, say so. If you avoid family, say why and since when.

B6.3 "You go to AA and a weekly social outing — tell me about those." HIGH

"How often do you go? Do you talk to people there? Do you have a sponsor? Do you see anyone from AA outside meetings? Who picks the weekly outing, and what happens there?"

GUIDANCE Describe it exactly. How often you actually go (not how often you're supposed to), whether you speak, whether anyone there knows you, whether you leave early, whether Erica brings you, whether the outing is a therapy assignment you force yourself through, and what happens afterward (shutdown, panic). If it's genuinely a relationship-building activity, say that too. This is the single fact a rater will use against total social impairment, so precision matters in both directions.

B6.4 **"What do you do for fun? Any hobbies? Video games, TV, exercise?"**

YOUR FILE Erica describes frequent all-night gaming; a Jan 2023 Starlite note says you'd stopped gaming "some years ago after an injury." The mock rating flagged an "active leisure life" note against anhedonia.

GUIDANCE Say what you actually do with your time and whether you enjoy it. If gaming is something you do alone at night because you can't sleep or to avoid thoughts, rather than for pleasure, describe it that way, and only if true. If something genuinely still gives you pleasure, it's fine to say so; one enjoyed activity doesn't erase depression.

B6.5 **"How often do you leave the house? For what? Can you go to the store alone?"**

GUIDANCE Count it: days out in the last week and why (appointments, AA). Say whether you go alone or with Erica, and what you avoid (crowds, stores, people from work). Your worksheet number goes here.

B7 · Mental-health course since service

B7.1 **"When did you first notice these symptoms, and how have they changed over time?"**

GUIDANCE Give the arc in a few sentences, in time order: 2012 changes in service → 2013–15 crash after separation (opioids, then benzodiazepines, panic, isolation, money) → treatment cycles from 2016 → the 2022 Starlite stay (where your signed statement says you first told a therapist) → therapy with Cherie from 2023 → worsening 2024–2026. Say whether the last six months are better, worse or the same, and how.

B7.2 **"What helps? Has therapy helped? Have medications helped?"**

ASSESSING Treatment response and prognosis — and, quietly, whether "symptoms are controlled" (a 10–30% concept).

GUIDANCE Be honest about partial benefits and their limits ("CPT helped me understand the self-blame; the nightmares, panic and isolation are still there"). Don't say "nothing helps" if something does.

B8 · Substance use — the SUD-secondary question

> The examiner must decide (1) whether you have an SUD, (2) whether it is secondary to the psychiatric conditions (self-medication) rather than primary, and (3) whether psychiatric symptoms can be distinguished from intoxication or withdrawal effects. Facts that help that analysis, **only if true**: when the symptoms started relative to the drug use; which symptoms persist during abstinence or residential stays; what you use the drug *for* (records: "helps me relax and feel less anxious," "to cope with work stress," "using to self-medicate"); and relapse triggers. Records also attribute relapses to "work stress" and being "recently laid off" — those can be symptom-driven too, but don't overclaim that every relapse traces to trauma.

B8.1 **"Tell me about your substance use from first use to now."** HIGH

"What substances? How much, how often, how do you get it? Longest period sober? IV use? Overdoses? What are you using currently, how much per day? When did you last use?"

YOUR FILE Pre-service kratom (18); escalating alcohol in service; post-service oxycodone/hydrocodone, then Valium/Xanax; heroin/IV use documented post-service (Victory 2018 onward); "14 months sober" ending 7/20/18; Victory IOP with clean toxicology; repeated detoxes; Suboxone (taper failed, maintenance from 5/4/26, stopped June 2026); relapse June 2026 on pain pills and kratom; current opioid use.

GUIDANCE Complete and honest, in time order. The examiner will compare your answer to the Victory and Starlite charts. State your longest sober periods and what your psychiatric symptoms were like during them. Say you are seeking treatment and what you've done so far.

TRAP Understating current quantity, or omitting heroin/IV history that is in the records.

B8.2 **"Which came first — the anxiety, depression and nightmares, or the drug use? When you're sober, are the symptoms still there?"**

GUIDANCE Answer from memory, with examples: during residential stays or the 14-month sobriety, were the nightmares, hypervigilance, panic, shame and SI present? If yes, say so concretely. Mittleider and Allen turn on exactly this information. If some symptoms do get worse with withdrawal, say that as well — accuracy is what makes the rest believable.

B8.3 **"What does using do for you?"**

GUIDANCE Describe the function honestly, in your own words (quiets the panic, lets you sleep, numbs the memories or the fear of failing — whatever is true). This is the self-medication mechanism Cherie's opinion relies on.

B9 · PTSD symptom review (DSM-5 Criteria B–G)

Examiners usually walk the PCL-5 or CAPS-5 structure: for each symptom, **how often in the past month**, **how intense**, and **an example**. Have a recent example ready for each one that's true. "Past month" is the default window — if something is constant over years, say so.

B9.1 **Intrusions (Criterion B): "Do you have unwanted memories of the event? Nightmares? Flashbacks? Do reminders upset you, physically or emotionally?"**

"How many nights a week do you have nightmares? What wakes you? What reminders set you off — parties, women who resemble her, alcohol, certain settings?"

GUIDANCE Give frequency, content (in general terms), duration and aftermath (can't get back to sleep, sweating, the next day ruined). Name your actual triggers.

B9.2 **Avoidance (Criterion C): "Do you avoid thinking or talking about it? Avoid people, places, situations that remind you?"**

GUIDANCE Give concrete examples. Note that the ten years of not disclosing is itself avoidance (it ties B4.4 together). Include the avoidance of social settings that drives the isolation.

B9.3 **Negative thoughts and mood (Criterion D): "Do you blame yourself? Feel you're damaged? Distant from people? Unable to feel positive emotions? Lost interest in things?"**

YOUR FILE CPT impact-statement themes: self-blame, "damaged in a way that can't be fixed," trust, intimacy, esteem.

GUIDANCE These are often your most persistent symptoms — describe them as they actually run day to day. If the past is hazy around the event, say so (dissociative amnesia is a Criterion D item), but only if true.

B9.4 **Arousal (Criterion E): "Irritability or anger outbursts? Reckless behavior? Always on guard? Easily startled? Concentration? Sleep?"**

"How often do you lose your temper? Ever broken things, hit anyone, been in fights? What does 'on guard' look like? How many hours do you sleep?"

GUIDANCE Hypervigilance: describe what you actually do (checking locks and windows, monitoring for anyone who might harm you or Erica, sitting facing doors) and how often. Anger: real frequency and worst examples, with no inflation toward violence. Sleep: hours, awakenings, medications. Recklessness: drug use and overdose risk count. Mention snoring or gasping only if Erica has observed it — it's relevant to a future sleep-apnea claim, not this exam's rating.

B9.5 **Duration and impact (F–G): "How long have these been going on? How do they affect work, relationships, daily life?"**

GUIDANCE Since 2012, worse after 2013, with the current level since 2024–26. Tie every symptom to a consequence. Symptoms with no consequence attached don't move the impairment box.

B10 · Depression, anxiety and panic, OCD

B10.1 **"How many days in the past two weeks have you felt down, depressed or hopeless? Interest in things? Energy, appetite, guilt, slowed down?"**

YOUR FILE DASS-21 (6/9/2026): Depression 36 / Anxiety 34 / Stress 38, all "Extremely severe" (taken about two weeks after a relapse — the examiner may note that).

GUIDANCE Use real counts, describe what a depressed day looks like hour by hour, and say how long the worst episodes last. "Near-continuous" is the 70% concept — if it's every day, say every day.

B10.2 **"Do you have panic attacks? Describe one. How often? How long? Triggered or out of the blue? What do you do?"**

YOUR FILE Cherie: "more mornings than not." Earlier corpus note: about 4–5 per week.

GUIDANCE Describe the physical sequence, frequency per week, duration and recovery time, and what you can't do afterward. Give the number from your worksheet. If the frequency has changed since Cherie's letter, say so and say when.

B10.3 **"How much of the day do you spend worrying? About what? Can you stop it?"**

GUIDANCE Name the content, the proportion of the day, and what it stops you doing.

B10.4 **"You've been diagnosed with OCD. What obsessions or compulsions do you have? How much time a day? What happens if you don't do them?"**

ASSESSING The 70%-tier symptom "obsessional rituals which interfere with routine activities." OCD is missing from your signed personal statement, so this exam may be the first full description.

GUIDANCE Give specific rituals (checking, counting, ordering, mental reviewing, reassurance-seeking — only what's real), minutes or hours per day, how they make you late or stop tasks, and how Erica experiences them. A concrete daily example beats a list.

B11 · Thought content, beliefs and perception

B11.1 **"Cherie's letter says you have 'paranoid delusions' about being fired. Tell me about that belief."** HIGH

"How certain are you, from 0 to 100? When people show you evidence you're doing well, what happens? Do you ever think it might be your anxiety? How much of the day do you think about it? What do you do because of it? Now that you're not working, how does it show up?"

ASSESSING Whether this is a **fixed, false belief held despite clear contrary evidence** (a delusion — 100%-tier language) or severe, evidence-resistant anxiety that you can sometimes recognize as excessive (70%-tier). The examiner, not you, makes that call.

YOUR FILE Cherie's letters describe your job-loss/failure fears as "paranoid delusions" and connect them to the overwork → collapse cycle. Your July account to the corpus described other belief content. Per your 18 Oct confirmation, the accurate description is the **fear of being fired / failing**, consistent with Cherie's letter. Both Recover evals (May 2026) recorded no delusions.

GUIDANCE Describe the belief exactly as you experience it: what you believe, how sure you are, whether reassurance or evidence ever moves it and for how long, how much of your day it occupies, and what you do because of it. Give real examples (employers showing you evidence of your value and the belief not budging, if true). **Don't use the word "delusion" yourself**, and don't describe beliefs you don't hold. If there are times you can step back and see it as probably excessive, say so — honesty here protects everything else.

TRAP Two failures: (a) self-labeling ("I'm delusional"), which reads as coached; (b) quickly agreeing "I know it's irrational" to seem reasonable when that isn't how it feels.

B11.2 **"Do you ever feel people are watching you, following you, or trying to harm you? Do you ever hear or see things others don't? Feel you have special powers or a special mission?"**

ASSESSING A standard psychosis and mania screen asked of everyone, plus a check against anything in your records.

GUIDANCE Answer each part truthfully and narrowly. Distinguish hypervigilance (scanning for threats) from believing a specific person is after you, and describe only what you actually experience. The corpus contains earlier descriptions of other beliefs, so the most important thing is that what you say Wednesday is true and consistent with Cherie's record. If you're unsure how to describe something, describe the experience and let the examiner characterize it.

B12 · Safety: suicidal and homicidal ideation, violence, means

B12.1 **"Do you have thoughts of death or of killing yourself? How often? Any plan, intent, or preparation? Any past attempts?"** HIGH

"Passive — wishing you wouldn't wake up — or active? When was the last time? What stops you? Any overdoses — accidental or intentional? Your records show you denied SI several times this year; help me understand."

YOUR FILE 5/14/26 FMLA eval: "denies current SI … history of passive SI." 6/17/26 personal statement: SI "every day." ~6/25/26 chart: "denies SI." You (July 2026): passive SI about twice daily (morning and night) plus episodes triggered by panic or shame; Erica's 7/12 statement also addresses SI — re-read it so your numbers match hers. Cherie: "suicidal thoughts daily"; you denied SI at inpatient admission "to avoid being placed on involuntary hold." Starlite ASAM Dimension 3 has a checkbox suggesting self-mutilation or a suicide attempt, which may be template text — never confirmed.

GUIDANCE Give your real current pattern: frequency, passive vs. active, triggers, duration, and what keeps you safe. Explain the "denies" entries in one or two sentences: you denied it at intakes to avoid a hold. Answer plainly whether you have ever attempted or overdosed — intentionally or not. If asked about the ASAM entry and it isn't accurate, say so. Daily passive SI is serious and ratable (*Bankhead*); it does not need to be described as more than it is.

TRAP Denying SI now to seem stable, which repeats the exact conflict in your file. And if you are in danger, the examiner is required to act. That's okay — your safety comes first.

B12.2 **"Any thoughts of hurting anyone else? Any violence, arrests, protective orders since service? Do you have access to firearms?"**

GUIDANCE Answer truthfully and specifically. If you have firearms at home, say so and say how they're secured. Examiners ask every veteran with SI.

B13 · Daily functioning, cognition, competency — and the closing

B13.1 **"Walk me through a typical day, from waking up to going to sleep."** HIGH

"What time do you get up? Who gets you up? Do you shower, brush your teeth, dress yourself? Who cooks? Who handles bills, medications, appointments? What do you do between noon and dinner?"

ASSESSING Everything at once: ADLs, motivation, structure, social contact, concentration, function.

GUIDANCE This is the most important open question in the exam. Describe a **typical** day and then a **bad** day (and how many bad days per week). Narrate it hour by hour, naming who does each task. Include what Erica does: prompts or helps you shower, reminds you to brush your teeth, picks out and washes your clothes, feeds you, cuts your hair, manages medications, money and appointments, and comes to appointments because you can't follow them — as far as each is true and as often as it happens.

B13.2 **"How often do you go without showering or brushing your teeth if no one reminds you? What would happen if your wife were away for a week?"** HIGH

YOUR FILE WHODAS self-care 62.5%, with severe difficulty "staying by yourself for a few days." Erica's statement. Cherie's letter (hygiene, clothing, appointments).

GUIDANCE Answer with your worksheet numbers and a real example (a time she was away, or a bad stretch). "Intermittent" inability is the regulatory word, so recurring bad stretches count. Describe the actual pattern; it doesn't need to be every day.

B13.3 **"How is your memory and concentration? Do you forget appointments, conversations, things on the stove? Can you read a book, follow a show, finish a task?"**

GUIDANCE Give concrete examples and say how Erica compensates (reminders, calendars). Keep it at its true level. Forgetting appointments is real impairment; memory loss for your own name or relatives' names is something else, and not your experience.

B13.4 **"Who manages your money? Could you manage VA payments on your own?"**

GUIDANCE Answer truthfully (see A8 for the fiduciary consequence). If Erica controls finances because of drug spending, say so.

B13.5 **"Is there anything else you'd like me to know? Anything I didn't ask about?"** HIGH

ASSESSING Your last chance to make the record complete.

GUIDANCE Use it. Glance at your notes page and add anything not covered, such as: the hygiene/ADL help if not discussed; how today compares with a typical day; frequencies the examiner didn't ask for; the reason for the "denies" entries; OCD rituals; that Erica can describe what she sees, if they'll hear from her. One or two sentences each. "I'm used to holding it together for appointments like this; this isn't what most of my days look like" is useful *if true*.

B14 · Additional probes examiners commonly use (rehearse a truthful answer to each)

History

1. Highest education? Grades? Any trouble finishing your degree?
2. Siblings? Relationship with your brother and parents now?
3. Any prior marriages or long relationships? How did they end?
4. Children?
5. Family history of mental illness or addiction?
6. Any head injuries or loss of consciousness?
7. Medical conditions and medications (Eliquis/PE history)?
8. Chronic pain? The ankle now?
9. Why did you separate in 2013? Did you consider reenlisting?
10. What did you do the first year after separation?
11. How did you pay for things when the savings ran out?
12. How many jobs since 2016? Longest held? Why each ended?
13. Were you ever disciplined at a civilian job?
14. How did you get the CTO offer? What happened with it?
15. What were you doing during the 2018 IOP and why were you discharged? (non-attendance)
16. Any legal trouble since service — DWIs, possession, theft?
17. Any previous VA claims? What happened? (2024 denial: records not obtained)
18. Who referred you to Cherie? Why did you start therapy in 2023?
19. When did you first tell Cherie about the assault?
20. Why did you participate in EMDR in 2022 if you denied trauma?
21. Did you complete the Harmony Junction program? Zoe PHP?
22. What were you treated for at Ocean's in 2025?
23. Who wrote your personal statement? Did anyone help? (Truthful: your own account; editorial help only, if that's accurate.)
24. How do you know Reyes and Poquette? When did you last talk to them? Did you ask them what to write? (Truthful answer — witnesses wrote their own.)

Current symptoms & function

25. How many hours a night do you sleep? Naps? Sleep medication?
26. Appetite and weight changes in the last year?
27. Crying spells — how often?
28. Do you feel hopeless about the future? Worthless?
29. Rate your mood 0–10 on a typical day; on a bad day; today.
30. Do you have energy to do chores? Who does them?
31. Do you drive? Any accidents or near-misses? (parking-garage incident in Erica's statement, if asked)
32. Can you go to a grocery store alone? A restaurant?
33. Any panic in the car, in lines, on the phone?
34. Do you answer the phone or the door?
35. How do you react to sudden noises?
36. Do you check doors and locks? How many times?
37. Do you feel detached from your body or surroundings?
38. Do you have trouble trusting people? Women specifically?
39. Any problems with intimacy related to the trauma?
40. How do you handle criticism or conflict?
41. When were you last angry? What happened?
42. Do you feel guilt or shame most days? About what?
43. What are your goals? What would you do if you were well?
44. What does your wife say you're like now versus when you met?
45. Do you think you could work part-time, from home, or with no deadlines?
46. How long can you focus on a task before you stop?
47. What happens to you in the days after a stressful event?
48. Have you ever been told you're a danger to yourself — any holds or hospitalizations for SI?
49. Do you keep naloxone? Ever been revived from an overdose?
50. Are you willing to engage in treatment? What's stopping you right now?

Count: 50 annotated question blocks (~100 examiner questions) with ~70 follow-up probes, plus the 50 additional probes above — over 200 rehearsal prompts in all.

Part C — The Questionnaires

What each instrument measures and how to take it honestly. Item wording is paraphrased; you'll see the real forms on Wednesday.

> **One rule for every questionnaire:** read each item, think about the time window it asks about (usually "past month" or "past two weeks"), and answer that item truthfully. Don't answer by pattern ("all 4s"), don't skim, and don't soften because answering feels dramatic. Psychometric scores never stand alone as a diagnosis or rating — they corroborate the interview — but an invalid profile can be used to discount everything.

Instrument	What it is	Scoring (for context)	Notes for you
PCL-5 (PTSD Checklist for DSM-5)	20 items covering the four DSM-5 PTSD clusters — intrusions (5), avoidance (2), negative thoughts/mood (7), arousal/reactivity (6). Each rated 0 "not at all" to 4 "extremely" for the **past month**. Public domain (National Center for PTSD); the most common PTSD measure at C&P.	Total 0–80; roughly 31–33+ suggests probable PTSD. Cluster scores map to Criteria B–E.	Answer about how much each problem **bothered** you in the past month — not whether it ever happened. Several items are about the "stressful experience," so keep the 2012 event in mind as the reference event.
CAPS-5 (interview, sometimes)	Clinician-administered gold-standard PTSD interview. For each symptom: frequency and intensity, with examples.	Clinician rates severity 0–4 per item.	This is what B9 rehearses. Have a recent example for each symptom you have.
PHQ-9	9 depression items for the past two weeks (interest, mood, sleep, energy, appetite, guilt, concentration, slowing or restlessness, thoughts of death), each 0–3.	0–27; 20+ severe. Item 9 asks about thoughts of death or self-harm.	Item 9 should match what you say in the interview. A "0" on item 9 next to daily passive SI in your statements recreates the exact conflict in your file.
GAD-7	7 anxiety items for the past two weeks, each 0–3.	0–21; 15+ severe.	Same discipline: two-week window, real frequency.
C-SSRS screen (Columbia)	Structured suicide-risk questions: wish to be dead; active thoughts; method; intent; plan; preparatory behavior; lifetime and recent attempts.	Risk level drives the examiner's safety steps.	Answer each question literally and truthfully (B12.1). "Wish to be dead" and "active thoughts of killing yourself" are separate questions — say which applies.
OCD measures (Y-BOCS or OCI-R checklist, sometimes)	Time occupied, interference, distress, resistance and control for obsessions and compulsions.	Y-BOCS 0–40; 24+ severe.	Have your time-per-day estimate ready (B10.4).

Symptom-validity tests	Possible examples: **MMPI-2-RF / MMPI-3** (long true/false personality inventory with over-reporting and inconsistency scales), **SIMS** (75 true/false items about unusual or implausible symptoms), **M-FAST** (brief interview screen for feigned psychiatric symptoms), **TOMM** (picture-recognition memory test where genuine patients score near-perfectly).	They flag patterns: endorsing rare, implausible or contradictory symptoms; random or inconsistent answers; under-performance on easy effort tasks.	**Answer exactly what's true.** Many items describe odd or extreme experiences that most genuine patients don't have — leave them unendorsed unless they really describe you. On memory and effort tests, just do your honest best. Research shows genuine PTSD and depression can raise some validity scores, which is why careful, item-by-item honesty matters more for you, not less. Don't fill these out while impaired or in withdrawal — inconsistent responding is scored.

Your existing instruments: DASS-21 (6/9/2026) Depression 36 / Anxiety 34 / Stress 38, all "Extremely severe" (98th–100th percentile); WHODAS 2.0 (6/11/2026) about 70% overall disability — self-care 62.5%, getting along 70%, life activities 75%, participation 75%. Both are self-administered, vendor-scored ("Recover") instruments, so describe them that way if asked. Examiners commonly give fresh measures rather than relying on old ones.

Part D — Two Mock Examiner Reports, Same Veteran

Illustrative only. Not a prediction, and not language to repeat. They show how the same facts become different checkboxes depending on what gets said and asked.

Read these side by side. Report A is what happens when you answer politely and briefly, have a better-than-average day, Erica isn't heard from, and the examiner doesn't dig. Report B is what happens when the *same true facts already in your file* are actually elicited, with frequencies and consequences. Report B uses only facts already in your record or that you have reported. Everything else is a **[bracket]** for you to fill with the truth — or delete if it isn't true. The brackets are not model answers.

Report A — the thin exam (→ likely 50–70%)

SECTION I — DIAGNOSIS

1. PTSD (F43.10) — criteria met by self-report. 2. Opioid use disorder, severe (F11.20), current use. 3. Major depressive disorder, recurrent, moderate (F33.1). 4. Generalized anxiety disorder (F41.1). OCD: "history of," insufficient current symptoms reported to confirm.

Differentiation: Not possible to fully differentiate; "Veteran's ongoing opioid use likely contributes substantially to mood and anxiety symptoms."

> Two quiet harms: OCD drops out (it was never described), and the active-use sentence invites the rater to discount symptoms as substance-induced. Prevented by B10.4 and B8.2 answers.

Level of occupational and social impairment:

☐ Total occupational and social impairment
☐ Deficiencies in most areas
■ Reduced reliability and productivity

SECTION II — CLINICAL FINDINGS (excerpt)

Social: "Married since 2022; describes wife as supportive. Attends AA meetings and reports a weekly social outing. Occasional contact with service friends."

> Each sentence is true and each one reads as preserved social function, because nothing about caregiving, reciprocity or what those contacts actually are was said.

Occupational: "Holds a B.S. in computer science; worked in technology roles, most recently left a position in 6/2026. Veteran reports he chose to leave the tech field."

Stressor: "Reports in-service sexual assault in 2012. Did not report. Records note denial of MST at 2015 VA screening and denial of trauma at multiple treatment intakes. Veteran did not offer an explanation."

SECTION III — SYMPTOMS (checked)

■ Depressed mood ■ Anxiety ■ Chronic sleep impairment ■ Panic attacks more than once a week ■ Disturbances of motivation and mood ☐ Suicidal ideation ☐ Obsessional rituals ☐ Intermittent inability to perform ADLs

Suicide risk: "Endorses passive thoughts 'sometimes'; denies plan/intent. Prior records variously document denial of SI."

BEHAVIORAL OBSERVATIONS (video)

"Casually dressed, adequately groomed, cooperative, good eye contact, speech normal, mood 'okay,' affect constricted, thought process linear, no evidence of delusions or hallucinations, insight and judgment fair."

> "Adequately groomed" with no explanation directly contradicts Erica's statement — and the rater may resolve the conflict toward the examiner's observation. "Mood 'okay'" came from B0.2.

REMARKS / OPINION

"PTSD at least as likely as not related to reported MST; markers include behavioral change per lay statements. SUD: less likely than not secondary — veteran reports substance use beginning at age 18 prior to service, and records attribute relapses to work stress."

Report B — the complete exam (→ supports 70–100%)

SECTION I — DIAGNOSIS

1. PTSD (F43.10), chronic, related to in-service personal assault (MST). 2. Major depressive disorder, recurrent [severity per examiner]. 3. Generalized anxiety disorder. 4. Obsessive-compulsive disorder (F42.2) — checking/[rituals as described] [n] hrs/day, interfering with routine activities. 5. Opioid use disorder, severe, current use — assessed as secondary to PTSD/MDD/GAD (self-medication; onset of disorder-level use post-dates the 2012 stressor; persistence of nightmares, hypervigilance, panic, and SI during periods of abstinence including residential treatment [as veteran reports]).

Differentiation: "It is not possible to differentiate symptoms or impairment among these disorders without resort to speculation; they are considered together."

> This is the Mittleider posture: inseparable symptoms are attributed to the service-connected picture. It came from facts the veteran supplied in B8.2.

Level of occupational and social impairment:

■ Total occupational and social impairment
☐ Deficiencies in most areas
☐ Reduced reliability and productivity

SECTION II — CLINICAL FINDINGS (excerpt)

Social: "Married (11/2022); spouse present with veteran's consent. Per veteran and the record, spouse functions primarily as caregiver: prompts or assists showering and toothbrushing [n] days/week, selects and launders clothing, prepares meals and prompts eating, cuts his hair, manages medications, finances and appointments, and attends appointments because he cannot track them. Couples counseling intermittently since 2020; periods of near-divorce. Outside the marriage: [describe any friendships and whether they are reciprocal]; last non-claim contact with a friend [date]; family contact [describe]; attends AA [n]x/month [how he gets there; whether he speaks or knows anyone]; the 'weekly outing' [what it is, whose idea, what happens after]. WHODAS 'making new friends' = cannot do."

> Same marriage, same AA, same outing — now described as what they actually are. Only accurate if each bracket is filled with the truth.

Occupational: "Employment marked by a cycle of fear-driven overwork followed by collapse and relapse: near job loss 2018; job loss 2023 (~6 months unemployed); laid off 12/2024; 6/2026 performance plan and negotiated severance after FMLA leave and residential treatment; unable to sustain work since. Veteran does not believe he could sustain even simple work because of [hygiene/getting up, panic, concentration, fear of failure]."

Stressor: "Incapacitated sexual assault by a coworker's (civilian) spouse, summer 2012 (meets Criterion A). Veteran states the 21-0781 entry 'service member' was an error. Non-disclosure at 2015 VA screening and treatment intakes explained as shame, not conceptualizing the event as assault, distrust of intake staff, and avoidance of involuntary holds — consistent with

documented patterns of delayed disclosure in male MST survivors. Markers: behavioral change in 2012 (Reyes: summer 2012; Poquette: ~8–9 months after meeting in 2011), escalating alcohol use and late-2012 alcohol-related discipline, in-service insomnia treatment [if STRs confirm], withdrawal from peers, post-separation financial collapse."

SECTION III — SYMPTOMS (checked)

■ Depressed mood ■ Anxiety ■ Chronic sleep impairment ■ Panic attacks more than once a week ■ Near-continuous panic or depression affecting ability to function independently ■ Suicidal ideation (passive, ~twice daily, panic/shame-triggered; no current plan or intent) ■ Obsessional rituals interfering with routine activities ■ Difficulty adapting to stressful circumstances ■ Inability to establish and maintain effective relationships ■ Intermittent inability to perform activities of daily living, including maintenance of minimal personal hygiene ☐ Persistent delusions — *examiner's determination*: fixed belief he will be fired / fail, held at [n]% certainty despite contrary evidence (employer reassurance), occupying [n] hrs/day, driving compulsive overwork.

> The "?" is honest. Whether the fear of being fired/failing is a delusion is the examiner's clinical call. Your job is to describe it precisely (B11.1). Report B reaches "total" even without that box — on ADLs + total social + total occupational impairment.

BEHAVIORAL OBSERVATIONS (video)

"Groomed for the appointment [with spouse's assistance — per veteran, if true]. Psychomotor restlessness, intermittent poor eye contact, tearful when discussing the stressor, mood 'terrible / exhausted,' affect anxious and dysphoric."

REMARKS / OPINION

"PTSD at least as likely as not caused by in-service personal assault. MDD/GAD/OCD at least as likely as not caused or aggravated by the same. Veteran's self-report of anxiety/depressive symptoms and kratom use at ~18 noted; pre-service functioning (enlistment, BMT Honor Graduate, sustained duty performance, intact peer network) contrasts with marked post-2012 decline. SUD at least as likely as not secondary to PTSD/MDD/GAD as maladaptive coping. Veteran's competency to manage VA funds: [examiner's finding]."

Part E — Checklist, Debrief, Rubric, and After

Logistics first, then self-scoring, then the post-exam playbook.

E1. Before Wednesday

☐ Confirm the appointment time and video link with VES (877-637-8387); confirm your phone number on file.

☐ Test camera, microphone and internet in the actual room; charger plugged in; second device ready.

☐ Photo ID; appointment letter; medication list (names, doses, prescriber).

☐ Fill in the frequency worksheet (notes doc) with Erica.

☐ Re-read your own signed statements once (personal statement, 21-0781), plus Cherie's and Erica's letters, for consistency.

☐ Decide with Erica: she sits in only if the examiner agrees; she doesn't answer for you; she can add collateral at the end if invited.

☐ Plan for safety: naloxone accessible; 988 → press 1.

☐ Plan to be neither impaired nor in unmanaged withdrawal; tell the examiner honestly at the start if either applies.

☐ Ask your VSO (today or Monday): is everything submitted actually *in* the claims file — Cherie's updated letter, Erica's 7/12 statement, Reyes, Poquette, DASS-21/WHODAS? Was the 21-0781 perpetrator correction filed?

☐ Sleep apnea: confirm whether an Intent to File (21-0966) was ever filed. If not, file one now — it costs nothing and locks an effective date.

E2. During the exam

- Log in 10–15 minutes early. Quiet, private room, door closed.
- Answer "How are you?" truthfully.
- Give numbers, durations and consequences — a real example from the last month for each.
- Typical day and bad day — and how many bad days.
- Correct errors calmly (perpetrator status; "service member").
- If you don't know or don't remember, say so. Don't fill gaps.
- If the examiner asks Erica to leave, agree.
- If you need a break, ask for one. Don't push through a panic attack silently — tell the examiner it's happening.
- Use the closing question (B13.5) to add anything missed.

E3. Red flags to note afterward

- Exam under ~30 minutes with no testing.
- No questions about ADLs, social life outside the marriage, SI, or OCD.
- Examiner says or implies non-reporting means it didn't happen.
- No discussion of the records or of Cherie's opinion.
- Examiner refuses to consider symptoms because of substance use without analysis.

E4. Self-scoring rubric — use during rehearsal and in the debrief

Did the exam (or your rehearsal) capture…	Yes	Partly	No
A truthful answer to "How are you?" and grooming explained (Erica's help)	☐	☐	☐
Current substance use disclosed honestly, plus symptoms during sober periods and treatment-seeking	☐	☐	☐
Pre-service history answered without contradiction (age 18, kratom, arrests, childhood)	☐	☐	☐
Stressor stated consistently (summer 2012; coworker's wife; incapacitated) and the 21-0781 error corrected	☐	☐	☐
Non-disclosure explained in your own words (2015 / 2018 / Starlite)	☐	☐	☐
Each PTSD cluster with frequency and a recent example	☐	☐	☐
Panic frequency, depression days and OCD time per day with numbers	☐	☐	☐
Fear of being fired/failing described (certainty, evidence-resistance, time, behavior) without self-labeling	☐	☐	☐
SI: real frequency and nature, plus why records say "denies"	☐	☐	☐
ADLs/hygiene: who does what, how often, what happens without Erica	☐	☐	☐
Social prong: every relationship outside the marriage described exactly (AA, outing, friends, family)	☐	☐	☐
Occupational sequence with mechanism (2018 → 2023 → Dec 2024 → June 2026) and current capacity	☐	☐	☐
Competency / money answered truthfully	☐	☐	☐
Closing question used to fill gaps	☐	☐	☐

E5. Debrief — write this down within an hour of the exam

Start and end time · examiner's name and credentials · whether Erica was allowed to stay or speak · which questionnaires you took · topics covered and **not** covered (use the rubric) · anything the examiner said about records, non-disclosure or substance use · anything you said that you now think was inaccurate or incomplete. Send it to your VSO.

If you realize you misstated something, tell your VSO. A short, truthful clarifying statement (VA Form 21-4138) is far better than leaving an error in the record.

E6. After the exam

1. **Get the DBQ.** Exam results usually appear in your VA.gov claim file or C-file over the following days or weeks. Ask your VSO to pull it, or request your claims file.
2. **Read it against the rubric.** If it is inaccurate or incomplete (missing ADLs, social, OCD or SI; treating non-reporting as negative evidence; dismissing symptoms as substance-induced with no analysis; ignoring Cherie's opinion), your VSO can submit a written response *before* the decision, pointing to *Barr v. Nicholson* (an exam VA provides must be adequate), *AZ v. Shinseki* (missing report ≠ negative evidence), 38 CFR 4.126(a) (rate on the whole record), and

Mittleider (inseparable effects go to the service-connected condition). A treating-provider DBQ from Cherie or a doctoral-level clinician can counterweight a thin exam.

3. **At the decision:** calendar the one-year deadline immediately (Supplemental Claim, Higher-Level Review, or Board appeal — your VSO picks the lane). Under *Rice*, VA may also consider TDIU because unemployment is in the record. You don't have to request it, and it wouldn't stop the schedular argument.
4. **Treatment.** Whatever happens with the claim, getting into SUD and mental-health treatment is the priority, and it also documents the condition going forward.
5. **Sleep apnea.** Still unfiled and no study yet. ITF now; study when you can; file inside the ITF year.

> **THE WHOLE EXAM IN ONE SENTENCE**
>
> Tell the truth at its real size: how often, how long, how bad, what it stops you doing, and who has to do it for you — and fix the few known contradictions calmly rather than letting the examiner find them.

Sources and legal references

38 CFR 4.130 (General Rating Formula; 2022 proposed revision RIN 2900-AQ82, 87 FR 8498, not finalized as of the latest available check, Aug 2026) · 38 CFR 4.126(a)–(b) · 38 CFR 4.25 · 38 CFR 4.16(a) · 38 CFR 3.102 · 38 CFR 3.301(c)(3) · 38 CFR 3.304(f)(5) · 38 CFR 3.353 and Part 13 · 38 CFR 3.655 · 38 U.S.C. § 1111 · *Mauerhan v. Principi*, 16 Vet. App. 436, 442 (2002) · *Vazquez-Claudio v. Shinseki*, 713 F.3d 112 (Fed. Cir. 2013) · *Bankhead v. Shulkin*, 29 Vet. App. 10 (2017) · *Mittleider v. West*, 11 Vet. App. 181, 182 (1998) · *Allen v. Principi*, 237 F.3d 1368 (Fed. Cir. 2001) · *AZ v. Shinseki*, 731 F.3d 1303 (Fed. Cir. 2013) · *Barr v. Nicholson*, 21 Vet. App. 303, 311 (2007) · *Buchanan v. Nicholson*, 451 F.3d 1331 (Fed. Cir. 2006) · *Kahana v. Shinseki*, 24 Vet. App. 428 (2011) · *Rice v. Shinseki*, 22 Vet. App. 447 (2009). All citations checked by this repo's citation-verifier pass (18 Oct 2026; two corrections applied). Exam-practice sources: VA.gov "VA claim exam" page; VES FAQ and exam-preparation tips (ves.com); VA C&P Clinicians' Guide (MST chapter; psychometric testing); National Center for PTSD (PCL-5); peer-reviewed literature on SIMS/MMPI-2-RF/TOMM in veteran samples. Case facts: `va-disability/` corpus (COMPREHENSIVE-FINDINGS-2026-07, CLAIM-ASSESSMENT, issue register, nexus-letter evaluations, statement analyses, RATING-REPORT-REMEDIATION-PLAN), plus your 18 Oct 2026 status confirmations.